Panitumumab

Supportive-care review sheet · Individualise with the clinical team; not a prescription.

Day 1: panitumumab infusion. There is no home chemotherapy pump in this regimen. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Before treatment

Confirm the exact indication, validated extended RAS wild-type result, ECOG and benefit/risk, allergies and lung history. Check FBC, kidney/liver profile and magnesium, calcium and potassium.

Skin and nails

Use the local pre-emptive EGFR skin-care policy: moisturiser, sun protection, and prescribed topical treatment or antibiotics as indicated. Record severe episode number, doses withheld and recovery before restart; inspect for infection.

Minerals and follow-up

Monitor magnesium, calcium and potassium before and during treatment and for up to eight weeks afterwards. NCCP specifies monthly electrolytes for two months after panitumumab. Replace as indicated; there is no NCCP numerical automatic hold threshold.

Antiemetics and premedication

Panitumumab: low emetogenic risk. Use NCCP/local antiemetic guidance and an individual rescue plan. No routine panitumumab premedication; prior reactions may change the plan.

Bowel symptoms and hydration

Provide a written diarrhoea and hydration plan. Fever, blood, severe pain, reduced urine, dehydration or inability to drink require prompt assessment; a rescue medicine does not replace review.

Administration and access

Panitumumab is given in saline through a 0.22 micron filter and is incompatible with glucose. A peripheral cannula or existing line can be used; there is no home pump.

Medication and safety review

Do not combine panitumumab with bevacizumab-containing chemotherapy or IFL. Review all medicines and supplements with pharmacy. Discuss contraception and avoiding breastfeeding during treatment and for two months afterwards.

Before discharge

Use teach-back for skin care, symptoms requiring same-day or emergency help, contact numbers and the next review. Give the visual guide, treatment passport and SACT card. Eye pain/visual change, new lung symptoms and severe skin infection need prompt assessment.

> Skin table: after 1–2 withheld doses and recovery below grade 3, first/second/third severe episodes use 100%/80%/60% of original panitumumab dose; fourth episode or failure to recover requires discontinuation. Confirm each component separately.

Sources: NCCP 00225 v6 · Vectibix SmPC · NCCP antiemetic guidance